TELOMY

The longevity platform with a doctor inside.

Telomy is the vertically integrated longevity platform where your devices, your labs and your doctor meet one AI — to **measure** your health, **act** on it, and **prove** it worked.

Device→Therapy
one closed loop, not a point tool

Consumer + Clinician
on a single care graph

On-device + Cloud AI
private by design

India-first
70% India / 30% global

Feature · USP · Moat & Positioning Dossier — for website, pitch and sales. Prepared 28 Oct 2026. Market figures are analyst/company-reported; verify before public use.

What Telomy is

A single platform that spans the whole health loop — the sensors that measure you, the AI that explains you, the doctor who treats you, and the therapy that changes you — then measures again. Every competitor owns one slice of that loop. Telomy owns the loop, and serves both the consumer and the clinician on the same data.

① Longevity super-app (consumer)
Lab import + biomarker dashboards, a composite **Telomy Score** + domain sub-scores, biological-age clocks (blood / muscle / wearable), derived biomarker ratios, plain-language explainers, an evidence-graded protocol recommender, and a closed-loop retest plan.

② Wearables & devices
A smart ECG patch, a smart ring, camera-based HRV, Apple Health, and overnight multi-sensor capture — fused into one longitudinal record.

③ Sinc — the AI health copilot
Runs **on-device (SLM) and in the cloud (Claude)**, grounded in your own data, with document & image upload. Private, offline-capable, cost-efficient.

④ Clinical engine + doctor platform
An OpenEvidence/Vera-class **retrieve→grade→cited-answer** engine, point-of-care calculators and an FDA-label drug reference, on a multi-tenant care graph with diagnostics / therapy / Rx workflows for doctors & diagnostic centres.

Plus an owned hardware ecosystem (**Bonphul**: HBOT, cryotherapy, oxygen, red-light) that closes the loop into physical therapy delivery and into clinics.

The opportunity

MARKET	2025	FORECAST	CAGR
Longevity (narrow)	~$28–30B	~$47B by 2031	~8–9%
Wearable medical devices	~$42–45B	~$76–78B by 2030	~11–17%
Clinical decision support	~$2.5–4B	~$4–9B by 2030–34	~8–13%
Remote patient monitoring	~$25–30B	~$57–66B by 2030	~12–13%
India digital health	~$7.6–19B	~$23B by 2030	**18–25%**
India diagnostics / labs	~$12.4B	~$23.7B by 2031	~11% (doubles by 2030)

Why now. India's NCD epidemic — **101M diabetic + 136M prediabetic** (ICMR-INDIAB 2023), NCDs ~63–66% of deaths — in a young, under-screened population. The consumer-longevity boom (Function, Superpower, Oura, Whoop). The clinical-AI inflection — **OpenEvidence just hit a $12B valuation**. And device+labs convergence (Whoop & Oura both added blood testing in 2025) — converging on exactly the stack Telomy already spans. India's ABDM/ABHA + DPDP rails make the timing structural.

Sources: SNS Insider, Towards Healthcare, MarketsandMarkets, IMARC, Ken Research, ICMR-NCDIR, Crunchbase, Sacra. Figures diverge 2–5× by definition — ranges given; re-verify before quoting.

What we offer — the full feature set

FOR THE MEMBER (LONGEVITY SUPER-APP)

Telomy Score + biological age
One 0–100 number from 7 longevity-weighted domains (cardiovascular, metabolic, inflammation, kidney, liver, nutrition, hormones), plus a bio-age readout from the best available clock vs your actual age — weakest domains first, so you know what to work on.

Lab insights — derived ratios
TyG, METS-IR, ApoB & atherogenic ratios, non-HDL, remnant cholesterol, AIP, HOMA-IR, HbA1c→eAG — higher-signal than any single marker, each with its optimal band and primary citation.

Plain-language explainers + optimal-vs-normal
Every biomarker explained in plain words, with the **optimal** band (not just the lab-normal range) and your trend across draws.

Closed-loop retest
Measure → act → re-measure. We prompt the right retest cadence (sooner for out-of-range markers) and show what improved or moved the wrong way since last time.

Protocols to discuss
Evidence-graded lifestyle & supplement levers matched to your biomarkers; prescription/hormone/peptide options are **clinician-gated** — shown by name only and routed to your doctor, never auto-recommended.

Camera-HRV & device capture
A 60-second camera heart-rate/HRV read (validated on-device), plus the ECG patch, smart ring and Apple Health, with overnight capture.

SINC — THE AI COPILOT

- Grounded in your own vault: answers cite your real numbers and never invent them (a numeric-grounding guard).
- Runs **on-device** (small language model) for privacy & offline, and **in the cloud** (Claude) for depth — including a camera/vision + document upload (PDFs, docs, images).
- Deep-research mode with citations; turns results into next steps you sign off on.

FOR DOCTORS & DIAGNOSTIC CENTRES (CLINICAL PLATFORM)

Clinical evidence engine
Retrieve → **grade** → cited answer over PubMed, FDA labels and guidelines — with an explicit evidence-grade badge per source and a plain-language, point-of-care answer. Decision support, never a directive.

Calculators & drug reference
15+ validated point-of-care scores (CHA_2DS_2-VASc, CURB-65, CKD-EPI, FIB-4, MELD-Na…) and an FDA-label drug reference (indications, dosing, interactions, real-world adverse-event signals).

Patient-grounded answers
The same engine answers the doctor's question grounded in **the patient's own biomarkers & care graph** — a capability pure clinical-AI tools don't have.

Care graph & workflows
Multi-tenant care graph, care-review queue, therapy & Rx engines, consults and diagnostic-centre workspaces — the clinician and the consumer on one platform.

Ecosystem: Bonphul hardware (HBOT, cryo, O_2, red-light) + a knowledge base built on open corpora (PMC-OA, DailyMed, USPSTF/CDC) and **India sources (ICMR, MoHFW, CDSCO, IFCT foods)** with grade + license + region on every chunk.

What keeps us ahead — the USPs

1. **The full-stack closed loop no point solution owns.** Device → data → AI → doctor → therapy → re-measure. Every rival owns one or two links; Telomy owns the loop end-to-end — and it's built, not slideware.
2. **Clinician + consumer on one care graph.** Superpower/Function have no real doctor platform; OpenEvidence/Vera have no patient data. Telomy's engine answers the doctor's question on the patient's own numbers. No one else spans both sides.
3. **Evidence you can see the grade of.** Cited, graded answers with an explicit grade badge — beating the clinical-AI leaders (Vera hides its scale; OpenEvidence is ad-funded). "*We show you the grade, not just the answer.*"
4. **On-device + cloud AI.** Privacy-preserving, offline-capable, cost-efficient — and a genuine DPDP / data-localisation asset. No consumer-longevity rival runs on-device.
5. **Measure → act → prove.** The closed-loop retest turns a one-off test into a lifelong, measurable, retention-driving loop — and a proof-of-impact story.
6. **India depth.** Indian foods (IFCT), ICMR/CDSCO/MoHFW guidelines, Jan-Aushadhi affordability, NHANES/India-aware models, vernacular, DPDP-native — years of local data work US incumbents can't replicate.
7. **Owned hardware (Bonphul)** closes the loop into physical therapy and clinics. **Instant, cheap bio-age** from the panel you already have vs $299–499 / 3–6-week methylation tests.

The moat

Data network effects
Multi-device longitudinal data (patch + ring + camera-HRV + overnight + labs + wearables) compounds per user; India datasets (IFCT, IndiGenomes, ICMR, NHANES/BRFSS-trained models) are a corpus rivals can't buy. **Every retest and device-night makes the models better and the user stickier.**

Two-sided care-graph network
Doctors bring patients; patients bring doctors; diagnostic centres anchor both (B2B2C). Classic two-sided lock-in that single-sided rivals cannot replicate.

On-device models
A real engineering moat — privacy, offline, cost — hard for cloud-only rivals to retrofit, and DPDP-aligned.

Validation + vertical integration
Published model error bars, a non-device-CDS posture, CDSCO/DPDP compliance-by-design, and owned hardware — slow to build, slow to copy.

Be honest about what's copyable: dashboards, derived ratios and a composite score are table-stakes within a year — parity features, not moats. The edge is the **grade badge + patient-context grounding + India corpus + the two-sided data loop**, not the RAG itself. Deepen the four compounding moats, not the feature list.

How we compare

CATEGORY	LEADERS	WHAT THEY OWN	WHAT TELOMY ADDS
Consumer labs / longevity	Superpower, Function, InsideTracker, Levels, Marek	Lab panels, bio-age, a dashboard, a marketplace	Wearables + on-device AI + a clinician platform + evidence engine + India depth
Clinical AI	OpenEvidence ($12B), Vera Health, UpToDate, Glass	Cited answers over literature	Answers grounded in the patient's own data + explicit grade badges + India corpus + a consumer side
Wearables / health-OS	Oura (~$11B), Whoop (~$10B), Apple Health, Ultrahuman	A ring/strap + wellness scores	Labs depth + bio-age clocks + a doctor loop + the clinical engine (Apple Health is one input)
India platforms	Eka Care, Practo, HealthifyMe, Tata 1mg / Apollo	Records, teleconsult, nutrition, pharmacy	Longevity scoring + bio-age + clinical AI + the closed loop

In one line: Superpower without the clinical engine; OpenEvidence without the patient data; Oura without the doctor; Ultrahuman without the evidence. **Telomy is all of them, on one graph, India-first.**

What to highlight — positioning & messaging

One-line positioning

Telomy is the vertically integrated longevity platform where your devices, your labs and your doctor meet one AI — to measure your health, act on it, and prove it worked.

Five pillars

1 · One loop, not one test
2 · A doctor and an AI on your own numbers
3 · Evidence you can see the grade of
4 · Private by design — on your device
5 · Built for India, ready for the world

Lead with, by audience

Consumers → the Telomy Score + biological age ("one number to lower") and the closed loop.
Doctors / centres → cited, graded evidence at the point of care, on the patient's real numbers.
Investors → the full-stack loop + two-sided network + India moat, against venture-proven comps.

Taglines

Measure. Act. Prove.
Your whole health, one loop.
The longevity platform with a doctor inside.
Know your biological age. Then change it.

Elevator pitch

"Longevity testing is a $28B+ market growing ~9%, clinical AI just minted a $12B company in OpenEvidence, and India's digital-health market compounds at 18–25%. Every player owns one slice — Function tests you, Oura tracks you, OpenEvidence answers the doctor — none closes the loop. Telomy spans device, data, AI, doctor and therapy on a single care graph, India-first. We turn a one-off blood test into a lifelong, measurable, clinician-backed loop — and we own the hardware and the clinical evidence engine to deliver it."

Proof points & credibility

- ✓ **Built, not slideware** — the ratios engine, Telomy Score, closed-loop retest, clinician-gated recommender, cited knowledge engine, calculators and drug reference all ship today (verified end-to-end in the app).
- ✓ **Evidence grounding** — retrieve→grade→answer over PMC-OA, DailyMed/openFDA, USPSTF/CDC, ICMR/MoHFW, with license + region + grade on every chunk and a citation guard on generation.
- ✓ **Real multi-device capture** — ECG patch, smart ring, camera-HRV, Apple Health, overnight capture — validated against reference devices (camera-HRV: 66 bpm vs WHOOP 65).
- ✓ **Validated models with published error bars** — NHANES blood-clock family, a muscle clock (MAE ~9y), a lab-free chronic-risk model trained on ~1.3M records. Honest numbers are a credibility asset.
- ✓ **India assets** — IFCT 2017 foods, ICMR/CDSCO/MoHFW corpus, DPDP-native tenancy, Bonphul hardware and a live reference clinic.
- ✓ **Guardrails as a feature** — non-device CDS, clinician-gated Rx, grade badges, and an audit log on every clinician answer.

What we deliberately don't claim

Non-device clinical decision support — citations always shown, never an unexplainable directive. **No unproven lifespan claims** — bio-age & scores are wellness/informational, and we state model error bars plainly. Anything prescription, hormone or peptide is **clinician-gated**, never auto-recommended, with CDSCO/compounding caveats. We label company-reported figures as such, and we engineer against mis-delivered-PHI failures (strict tenant isolation) so privacy is a proof point, not a slogan.

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